

HOW TO OPEN A RECOVERY RESIDENCE (SOBER LIVING HOME) in Texas

Housing + peer support — no license required, but accreditation now decides your funding.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, fee & funding rule should be re-verified before you rely on it.

The Texas recovery-residence model — read this first

A recovery residence — a sober living home — is a safe, substance-free home where adults in recovery live together and support one another. It provides housing, structure, peer support, and accountability. It is NOT a treatment program, and that one distinction decides whether you need a state license. Texas does not license sober living homes, so anyone can technically open one — but House Bill 299 (2023) created Health & Safety Code Chapter 469, a voluntary accreditation system run by TROHN and Oxford House, and it changed the economics of the whole field. Since September 1, 2025, state-funded entities may put recovery-housing resources (rent assistance, bed fees) only into TROHN-accredited or Oxford House-chartered homes. Translation: no accreditation, no state funding and no state-linked referrals. This guide treats accreditation as the main event.

***Stay on the housing side of the treatment line.** The moment you add clinical services — counseling, therapy, detox, or managing residents' medications — you become a chemical-dependency treatment facility, which Texas licenses through HHSC under Health & Safety Code Chapter 464 (26 TAC 564). That is a completely different, heavier path. A recovery residence provides housing and peer support and CONNECTS residents to licensed providers for treatment — it does not provide treatment itself.*

***The five names to learn.** HHSC = Texas Health and Human Services Commission (licenses treatment, funds recovery housing, recognizes the accreditors). TROHN = Texas Recovery Oriented Housing Network, a NARR affiliate (one of two accreditors; certifies NARR Levels I–IV). Oxford House = the national self-run recovery-home network (the other accreditor; charters Level I homes). OSAR = Outreach, Screening, Assessment & Referral center (the state-funded “front door” that sends you residents). Be Well Texas = a UT Health San Antonio program that pays part of eligible residents' rent in accredited homes.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; fees, standards, funding rules, and statutes update regularly, so verify each step with HHSC, TROHN, Oxford House, and qualified local counsel (see Part 11) before you rely on it. Every statute and program named here is real and pulled from public Texas sources — but always check the current version.

The journey at a glance

Nine milestones take you from “I'm thinking about it” to a full, funded home.

1	Decide your model & level	Independent vs. Oxford House; NARR Level I–IV (Part 2)
2	Form the business & insure	LLC/nonprofit, EIN, bank account, insurance (Part 3)
3	Secure the home	A safe house, your zoning shield, fire & life safety (Part 4)

4	Write your policies	The Operations Manual — your rulebook & accreditation backbone (Part 5)
5	Get accredited	TROHN or Oxford House — the whole ballgame (Part 6)
6	Staff the home	House manager, screening, the voluntary-work rule (Part 7)
7	Set up the money	Resident fees, Be Well Texas subsidies, the ethics line (Part 8)
8	Fill your beds	OSAR & your referral network (Part 9)
9	Open & stay compliant	Costs, timeline, who to call (Parts 10–11)

Rule of thumb. Do the cheap, reversible things first (decide, form, lease) and start your accreditation application as early as possible — it's the longest pole in the tent. And don't count on state funding until you're accredited.

PART 1

What This Business Is

In short: A safe, substance-free home where adults in recovery live together and support one another — housing and peer support, NOT treatment. No license required, but accreditation now decides your access to state funding and referrals.

A recovery residence provides a stable, alcohol- and drug-free place to live plus the structure, peer support, and accountability that help people stay in recovery. Residents live together, follow house rules, participate in the recovery community, and support each other. What a recovery residence does NOT do is provide clinical treatment — no counseling as a service, no therapy, no detox, no managing residents' medications. That line is the single most important thing to understand, because crossing it turns your home into a licensed treatment facility overnight.

Housing vs. treatment — the line that defines your business

	Recovery residence (this guide)	Chemical-dependency treatment facility
What it is	Substance-free housing + peer support	Clinical treatment — counseling, therapy, detox
State oversight	No license; voluntary accreditation (Ch. 469)	HHSC license under Ch. 464 (26 TAC 564)
Who delivers it	A house manager + peers	Licensed clinical staff
Medications	Residents self-manage; you don't administer	Clinically managed

Stay on the left column. Connect residents to licensed providers for the treatment they need, but keep your home a home. If you ever want to add treatment, that's a separate HHSC Chapter 464 license and a different business.

Why accreditation now decides your future

Because Texas doesn't license sober living homes, anyone can technically open one — but House Bill 299 (2023) created Health & Safety Code Chapter 469, a voluntary accreditation system run by TROHN and Oxford House, and the September 1, 2025 rule changed the economics: state-funded entities may now put recovery-housing resources (rent assistance, bed fees) ONLY into TROHN-accredited or Oxford House-chartered homes. So while you can open without accreditation, an unaccredited home is locked out of state funding and state-linked referrals — which is why this guide treats accreditation as the main event, not an optional credential.

Key terms, in plain English. Recovery residence = a sober living home (housing + peer support). NARR = the National Alliance for Recovery Residences, whose Levels I–IV describe how much structure a home offers. TROHN = the Texas NARR affiliate that accredits homes. Oxford House = the national network that charters self-run Level I homes. Chapter 469 = the Texas accreditation law (HB 299). Chapter 464 = the treatment-facility licensing law (the line you don't cross). Chapter 123 = the zoning protection for community homes. OSAR = your state-funded referral front door. Be Well Texas = the rent-subsidy program. EIN = your federal tax ID. MAT/MOUD = Medication-Assisted Treatment — residents on it cannot be excluded.

Why this business — and why now

Demand for recovery housing in Texas is high and rising: treatment programs need somewhere to step clients down to, drug courts and reentry programs need stable housing, and hospitals and detox units need a safe landing spot. At the same time, Texas has built real funding and referral machinery around accredited homes — the Be Well Texas subsidies and the OSAR referral system — so a well-run, accredited home can fill its beds and get help paying for eligible residents. And it's a business a committed person can start lean: you don't need a license, a big clinical staff, or a consultant — you need a safe home, an honest operation, written policies, and accreditation.

Be clear-eyed about what makes a home succeed. The homes that fail usually do so for avoidable reasons: they open without pursuing accreditation and forfeit funding and referrals; they drift across the treatment line without a Chapter 464 license; they mishandle money (accepting or paying referral fees — a crime); or they can't keep beds full because they never built OSAR and referral relationships. Get accreditation, stay on the housing side of the line, run the money honestly, and build your referral network, and the demand does the rest.

PART 2

Decide Your Model and Level

In short: Pick independent vs. Oxford House, then pick the NARR level you can actually staff, then your population and size. This choice shapes every other step.

2.1 Independent home or Oxford House?

	Independent recovery residence	Oxford House
Control	You set rules, hire, choose population, any level	Follow the Oxford House charter
Staffing	Yours to build (a manager at Level II+)	Self-run, no paid staff (Level I)
Cost / overhead	Higher	Lower — residents share expenses
Best for	Operators who want to build a business	A proven, low-cost, resident-run start

2.2 Choose your NARR level

Level	What it looks like	Typical fit
I — Peer-Run	Residents self-govern; no paid staff (Oxford House model)	Self-directed residents further along
II — Monitored	At least one paid house manager; rules, testing, meetings	Early-to-mid recovery — most common new home
III — Supervised	Organizational staff & structure; life-skills programming	Residents needing more structure
IV — Service Provider	Clinical services + housing (needs an HHSC Ch. 464 license)	Higher-acuity / step-down

In practice. Most first-time operators open one Level II home for six residents — enough to be viable, small enough to run well, and aligned with the Chapter 123 zoning protection (Part 4). Never claim a level you don't actually operate; overstating it is a Chapter 469 ethics violation.

2.3 Pick your population and size

- **Population:** men, women (separate homes), young adults, or veterans — Be Well Texas subsidies specifically target young adults, so population affects your funding.
- **Capacity:** a single-family home commonly houses up to six residents — the Chapter 123 “sweet spot” that keeps your strongest zoning protection intact.
- **Staff to your level:** if you pick Level II, you must actually have a house manager; if Level III, real organizational structure. Pick the level you can genuinely staff and operate, not the one that sounds most impressive.

PART 3

Form Your Business & Insure — Step by Step

In short: A handful of mostly-cheap tasks that make you a real, bankable, insurable business. We break each one all the way down — especially getting your EIN and opening the bank account.

3.1 Form your legal entity

An “entity” is the legal business — usually a Limited Liability Company (LLC), or a nonprofit corporation if you plan to chase grants or a Be Well Texas partnership — that holds your lease, your contracts, and your bank account, and shields your personal assets. You form it with the Texas Secretary of State.

- 1. File your Certificate of Formation with the Texas Secretary of State.** An LLC (Form 205) filing fee is about \$300 — verify the current amount. Name the entity and list a registered agent (this can be you). Choose a nonprofit corporation with a board instead if you'll pursue grants or a Be Well Texas partnership.
- 2. Adopt your governing document.** An Operating Agreement for an LLC (or bylaws for a nonprofit) sets out ownership, management, and decision-making — your bank and your accreditor will want to see it.
- 3. Register with the Texas Comptroller for franchise tax.** Most small recovery residences owe little or no franchise tax, but you still file the annual report. Register at comptroller.texas.gov.
- 4. Check local business registration.** Some cities require a boarding/rooming-house permit above an occupancy threshold — that's the Chapter 260 question in Part 4. Confirm with your city before you sign a lease.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Do you need an NPI? (usually not)

An NPI (National Provider Identifier) is a number health-care providers use to bill Medicaid and insurance. A housing-and-peer-support recovery residence does NOT bill Medicaid or insurance for clinical services — your revenue is resident fees and subsidies — so most recovery residences do NOT need an NPI. The exception is if you become a Level IV “service provider” residence that adds clinical services under an HHSC Chapter 464 treatment license and bills for them; only then would you get a Type 2 (organizational) NPI at nppes.cms.hhs.gov. For a standard Level I–III home, skip it.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number

- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Texas branches — Chase, Wells Fargo, Bank of America, Frost Bank, or a local Texas credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A recovery residence houses people in a home you're responsible for, so you need real coverage — and your landlord, your accreditor, and any funding partner will want proof of it:

Coverage	What it protects against	Must-have?
General Liability	Injury or property damage at the home	Yes — foundational
Property / Contents	Fire and damage to the house and its furnishings	Yes
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Employment, discrimination, and governance claims	Recommended
Non-Owned & Hired Auto	If you or staff drive residents in a personal or house vehicle	If you transport residents

You are NOT buying malpractice. Because you don't provide treatment, you don't need clinical malpractice/professional-liability coverage the way a treatment facility does — buying the wrong (heavier) policy wastes money. Tell your agent clearly: this is recovery HOUSING with peer support, not treatment.

Real places to get it

Buy through an independent agent or broker who understands recovery housing / sober living specifically — a standard homeowner or small-business policy usually won't fit. Ask a fellow operator or your accreditor (TROHN) who they use, and find a local Texas agent at [trustedchoice.com](https://www.trustedchoice.com). Confirm the carrier will write a recovery residence in Texas before you rely on a quote.

What to ask the insurance agent

- "Do you write recovery residences / sober living homes in Texas — which carriers, and what's the minimum premium?"*
- "Is abuse & molestation coverage included, at what limit, and is it a sub-limit or a full limit?"*
- "Do I need non-owned/hired auto if my house manager sometimes drives residents?"*
- "What coverage limits does my landlord, my accreditor, or a Be Well Texas partnership require?"*

PART 4

The Home — Zoning, Fire & Life Safety

In short: *Pick a safe house in a real neighborhood, know your two legal protections, watch for the boarding-home permit trap, and pass a real fire and habitability check before anyone moves in.*

4.1 Your zoning shield (two protections)

- **Chapter 123 (Community Homes Act):** protects a qualifying community home (up to six residents plus two supervisors) from discriminatory local zoning — the city must treat it as a residential use.
- **The federal Fair Housing Act:** people in recovery are a protected disability class, so cities generally can't use zoning or occupancy caps to keep recovery homes out of residential areas.

Watch out — the boarding-home trap. *Separately from zoning, Chapter 260 lets counties and cities require a permit and inspections for “boarding homes” above certain occupancy. Whether it applies depends on your city and resident count — call your local code/permitting office and ASK before you sign a lease. This is the single most common surprise for new operators.*

4.2 Fire & life-safety — before anyone moves in

- ☐ Working smoke alarms in and outside sleeping areas, on every level
- ☐ Carbon-monoxide alarms near sleeping areas (with fuel appliances or an attached garage)
- ☐ At least one operational fire extinguisher
- ☐ Clear, unobstructed exits from every level; a written, practiced evacuation plan
- ☐ Locked, secured storage for medications, chemicals, and any firearms

4.3 Habitability — a safe, livable home

- ☐ Safe heat, water, and electrical systems
- ☐ A clean, sanitary kitchen and adequate real bedrooms (no sleeping in non-bedrooms)
- ☐ Lead-paint disclosure if the home was built before 1978
- ☐ A lease that allows your use and your resident count (if you're leasing)

What this means for you: the house is where accreditation and safety meet. A home that's clean, safe, and correctly zoned sails through the accreditation property review; one with an unresolved boarding-home permit or a fire-safety gap stalls. Handle the local permitting question and the safety checklist before you commit to a property, not after.

PART 5

Write Your Policies

In short: Accreditation requires written policies, and your Operations Manual is that rulebook. Adopt it, customize it to your home, and it becomes both your accreditation backbone and how you actually run the house.

TROHN and Oxford House both require written policies — on drug testing, prohibited substances, resident rules, intake, background screening, grievances, discharge, and medication. A Recovery Residence Operations Policy & Procedure Manual supplies all of it; you fill in your home's details. At minimum you need:

- **A written resident agreement:** fees, house rules, resident rights, grounds for discharge, and refunds — signed at intake so expectations are clear.
- **A substance-free policy and house rules:** clear, fair, graduated consequences rather than one-strike surprises.
- **A consistent drug-and-alcohol testing policy:** applied the same way to everyone, documented every time.
- **A recovery-focused relapse response:** relapse is handled with a safety focus and a plan — NOT automatic eviction.
- **MAT/MOUD non-discrimination:** you cannot exclude residents for using prescribed addiction medication (buprenorphine, naltrexone, methadone) — doing so is disability discrimination.
- **A grievance process and a fair, documented discharge procedure:** residents get a way to raise concerns and a fair, written process if they must leave.

Your policies ARE your accreditation. The written manual isn't busywork — it's most of what TROHN or Oxford House reviews, and it's what protects you and your residents when something goes wrong. Adopt it before you apply, and actually run the house by it.

PART 6

Get Accredited — TROHN or Oxford House

In short: *This is the whole ballgame after September 2025. Pick your body, submit the application, pass the review and property inspection, and keep it current — and start the day your policies are ready.*

6.1 Path A — TROHN (all four levels)

- 1. Create your account and start the online application.** It has four sections: orientation, provider information, policies/documents/attestations, and programs/properties.
- 2. Upload your policies and attestations.** Your Operations Manual (Part 5) and attestations; TROHN reviews them against the NARR + TROHN standards.
- 3. Respond to any flagged items.** It's a strength-based process — you can update and improve rather than simply pass/fail.
- 4. Pass the property inspection as applicable.** TROHN confirms the home meets the physical standards for your level.
- 5. Receive your accreditation and keep it current.** Renew on time; you can submit a variance request or a grievance if needed.

Budget for it. *TROHN's fee has been reported at roughly \$500 per application plus about \$10 per bed — confirm current fees directly with TROHN.*

6.2 Path B — Oxford House charter (Level I)

If you want a self-run, low-overhead home, Oxford House charters democratically governed Level I homes on its established model — residents share expenses and vote on house matters, backed by a national peer network. It's a distinct application and a distinct model from an independent home; you follow the Oxford House charter rather than building your own structure.

Do it from day one. *Start the accreditation application BEFORE you open, not after. A new home should be actively pursuing accreditation from the start — and an unaccredited home is locked out of state funding and referrals after September 1, 2025. It's the longest pole in your timeline, so start it the moment your policies are ready.*

PART 7

Staff the Home

In short: Staffing follows your level. Screen everyone in a position of trust, train the team on overdose response, and keep any resident work completely voluntary.

7.1 The roles

- **A house manager (Level II and up):** choose someone with real stability in their own recovery, sound judgment, and the ability to enforce rules fairly and kindly. The manager is the heart of a monitored home.
- **Background-screen everyone in a position of trust:** criminal-history background checks through the Texas Department of Public Safety for your manager and any staff.
- **Train the team:** overdose response and naloxone, resident rights, confidentiality, and — critically — the peer-support-vs-treatment line so no one drifts into providing “counseling.”

Keep resident work voluntary. Any resident work arrangement must be completely voluntary, with no penalty for declining. This is an explicit accreditation-standard rule — requiring resident labor is a violation.

7.2 Finding and keeping a good house manager

Your house manager makes or breaks a Level II+ home, so recruiting and keeping the right person is your core staffing job:

- **Recruit from the recovery community:** the best managers often come from alumni networks, mutual-aid meetings, and referrals from other operators — people with lived experience and stable time in recovery.
- **Hire for judgment and warmth:** the manager enforces rules AND supports people; check references for fairness and reliability, not just time sober.
- **Support and retain them:** a live-in manager especially needs clear boundaries, backup for time off, fair pay or reduced rent, and someone to call in a crisis. Manager burnout empties a house's stability fast.

PART 8

The Money — Fees, Subsidies & the Ethics Line

In short: Resident fees are your base; Be Well Texas subsidies and state funding reward accreditation; and paying or taking referral fees is a crime. Occupancy is everything.

8.1 Where the money comes from

Source	What it is	Notes
Resident fees / rent	Your baseline revenue	~\$150–\$250/week single-occupancy (verify locally); disclose all fees up front
Be Well Texas subsidies	HHSC-funded rent help in accredited homes	~\$4,500 (Level II) / \$6,800 (Level III) vouchers, 3–4 months, eligible young adults · 888-852-3935
State & grant funding	Chapter 469-linked dollars	Accredited/chartered homes only, since September 1, 2025

8.2 The money math — occupancy is everything

A recovery residence's economics are simple and driven by one number: how many beds are filled. Work it with your own figures:

- **Revenue:** a six-bed Level II home at \$200/week, full, grosses about \$1,200/week — roughly \$62,000/year — before expenses, plus voucher dollars for eligible residents.
- **Expenses:** rent or mortgage, utilities, the house manager, insurance, testing supplies, food (if provided), and accreditation. Most are fixed whether a bed is full or empty.
- **The lever:** every empty bed is lost revenue against fixed costs, so occupancy — filling and keeping beds filled through your referral network (Part 9) — is the whole game. A home that stays full is stable; one that runs half-empty bleeds cash even with low overhead.
- **What this means for you:** model conservatively (assume some vacancy and some residents who can't pay full fee), keep your fixed costs modest, and treat referral relationships as your most important business activity — because occupancy, not rate, is what determines whether the home survives.

Watch out — the crime. NEVER pay for a referral or accept payment for sending a resident to a provider or lab, in cash or in kind. That violates Chapter 469 ethics, the federal EKRA (Eliminating Kickbacks in Recovery Act, 18 U.S.C. § 220), and Texas solicitation law — and it can end your business and expose you to criminal liability. Fill beds through legitimate relationships, never kickbacks. If a lab or treatment center offers you money for residents, walk away.

PART 9

Fill Your Beds

In short: *Residents come through the OSAR system and your referral network. Start with your regional OSAR and 211, and build relationships ethically and consistently — occupancy is your survival.*

9.1 Where your residents come from

In Texas, residents arrive through a specific ecosystem: the regional OSAR centers (the state-funded front door), treatment providers stepping clients down, drug courts and reentry programs, hospitals and detox units, veterans' services, and recovery community organizations. Start with your regional OSAR and 211 Texas (dial 211, or 877-541-7905).

9.2 Your first-90-days referral plan

- 1. Get accredited first, then register with your OSAR.** Your regional OSAR is your single most important local relationship — call and ask exactly what they need to put you on their placement list.
- 2. Map your local referral sources.** List the treatment centers, drug courts, reentry programs, hospitals/detox units, and recovery community organizations in your area — these are the people who place people into homes.
- 3. Show up consistently and ethically.** Introduce yourself, share your accreditation and your one-page details, and be the home that answers the phone and admits smoothly — never with a referral fee.
- 4. Pursue a Be Well Texas subsidy partnership.** For eligible young-adult residents, a subsidy partnership both fills beds and helps residents pay — call 888-852-3935 to ask how to become a partner home.

The through-line is reliability and ethics: referral sources send people to the homes that are accredited, answer the phone, admit smoothly, communicate, and never ask for or offer a kickback. Your reputation with a handful of OSAR staff and treatment discharge planners is, in practice, your occupancy.

PART 10

What It Costs & How Long It Takes

In short: A few thousand dollars and a few weeks to open; the accreditation review runs weeks to a few months — so start it early. Don't count on state funding until you're accredited.

10.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Business setup	TX SOS LLC certificate of formation (~\$300), EIN (free), Comptroller registration, legal	~\$300 + legal
Insurance	General liability + property (+ abuse & molestation), annual	Get quotes
Accreditation	TROHN application + per bed (or an Oxford House charter)	~\$500 + ~\$10/bed (verify)
Background screening	DPS checks for your manager and staff	Per person; modest
Home setup	Furnishing, safety equipment, naloxone, testing supplies	Varies by home size
First month + deposit	Rent and deposit if leasing the home	Local market
Policies	The Operations Manual (one-time)	One-time

10.2 Timeline

Days	Business formed	LLC + EIN + bank account
Weeks	Home leased & set up	Furnished, safety-equipped, policies adopted, local permit question answered
Wks–Mos	Accreditation reviewed	TROHN/Oxford House application → review → property inspection → decision
Ongoing	Beds filling	OSAR + referral network; Be Well Texas subsidies for eligible residents

Rule of thumb. Start the accreditation application the day your policies are ready, and confirm current processing times directly with TROHN or Oxford House. Everything downstream — funding, referrals, subsidies — waits on accreditation.

PART 11

Who to Call & Exactly What to Ask

In short: Statewide contacts are the same for everyone; your OSAR and permitting are local. Here's the list and the exact scripts, so you get a straight answer instead of getting bounced around.

11.1 Statewide (same for everyone)

For...	Contact
Accreditation (all levels)	TROHN — trohn.org
Level I self-run charter	Oxford House — oxfordhouse.org
Recovery-home subsidies	Be Well Texas — 888-852-3935 · bewelltexas.org
State funding & provider info	HHSC Substance Use Services — hhs.texas.gov
Referrals / statewide help line	211 Texas — dial 211 or 877-541-7905
Crisis	988 Suicide & Crisis Lifeline — call or text 988

11.2 The local calls — and exactly what to ask

① Your regional OSAR / behavioral health authority — your most important local call

"I operate a recovery residence in [city] and I'd like to be a placement option for the clients you screen. What do you need from me to get on your referral list?"

"How do you document a housing referral, and do you require TROHN accreditation or an Oxford House charter?"

"Are there state-funded rent or bed-fee resources for residents, and how do they access them?"

② City / County Planning & Zoning — "Can I legally run a recovery home here?"

"I want to open a recovery residence — a home where up to [6] adults in recovery live together. Is that an allowed residential use at [address], or do I need a special/conditional use permit?"

"Does the Community Homes Act (Chapter 123) apply to my home? Are there occupancy or spacing rules? Can you confirm that in writing?"

"If they hesitate: homes for people with disabilities are protected as a residential use under Chapter 123 and the federal Fair Housing Act — please cite the specific code section you're relying on."

③ Local Code / Permitting — "Do I need a boarding-home permit?"

"Do you require a boarding or rooming-house permit for [6] unrelated adults living together, and if so, what's the application and inspection process? (This is the Chapter 260 question.)"

"What are your rules for smoke and carbon-monoxide alarms, exits, and any bedroom-egress requirements?"

④ Local Fire Department & ⑤ Your accreditor

"Fire: I'm opening a recovery residence — a private home for up to [6] adults. Any fire-code requirements beyond standard residential, and do you require an inspection or sign-off?"

"Accreditor (TROHN / Oxford House): What's the current application fee and per-bed cost, what documents do I submit, how long does review take, and what does the property inspection cover?"

11.3 Your regional OSAR / behavioral health authority — the numbers

Find your OSAR through 211, or start with the authority for your metro below. Numbers change — verify before you rely on them.

Metro (HHS region)	Behavioral health authority / OSAR	Phone (verify)
Houston (6)	The Harris Center	713-970-4400 (Option 6)
Houston (6)	The Council on Recovery	713-942-4100 · 855-942-4100
Dallas (3)	North Texas Behavioral Health Authority (NTBHA)	OSAR 844-275-0600 · crisis 866-260-8000
Fort Worth (3)	MHMR of Tarrant County	817-332-6329
San Antonio (8)	The Center for Health Care Services	OSAR 210-261-3076
Austin (7)	Bluebonnet Trails Community Services	OSAR 844-309-6385
Austin (7)	Integral Care (Travis County LMHA)	integralcare.org

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I need a license to open a recovery residence in Texas?

No — Texas does not license sober living homes, so you can open a housing-and-peer-support home without a state license. But since September 1, 2025, you need TROHN accreditation or an Oxford House charter to receive state funding or state-linked referrals — so in practice accreditation is essential even though a license is not.

What turns my home into a licensed treatment facility?

Adding clinical services — counseling or therapy as a service, detox, or managing residents' medications. That crosses into chemical-dependency treatment, which HHSC licenses under Chapter 464. Keep your home housing-and-peer-support and connect residents to outside licensed providers for treatment, and you stay on the housing side of the line.

Can I make residents who relapse leave immediately?

Your policy should treat relapse with a safety focus and a plan, not as automatic eviction — a recovery-focused response is an accreditation expectation. And you cannot exclude or evict someone simply for being on prescribed Medication-Assisted Treatment; that's disability discrimination.

Can a treatment center pay me to send them my residents?

No — never pay for or accept payment for a referral, in cash or in kind. It violates Chapter 469 ethics, the federal EKRA law, and Texas solicitation law, and it can end your business and expose you to criminal liability. Fill beds through legitimate OSAR and referral relationships only.

The seven mistakes that close recovery homes

Almost every recovery-residence failure traces back to one of these — and every one is avoidable:

- **Opening without pursuing accreditation:** after September 1, 2025 you forfeit state funding and state-linked referrals — start the application from day one.
- **Drifting across the treatment line:** providing counseling or managing medications without an HHSC Chapter 464 license turns your home into an unlicensed treatment facility.
- **Excluding residents on MAT/MOUD:** refusing someone on prescribed addiction medication is disability discrimination.
- **Accepting or paying referral fees:** an EKRA and Chapter 469 violation that can end your business and expose you to criminal liability.
- **Treating relapse as automatic eviction:** your policy should be a safety-focused response with a plan, not a one-strike removal.
- **Overstating your NARR level or accreditation status:** claiming a level you don't operate is a Chapter 469 ethics violation.
- **Requiring resident labor:** any resident work must be completely voluntary, with no penalty for declining.

A worked example — a Level II home in San Antonio

Here's the whole path in one metro. You form an LLC with the Texas Secretary of State and get your EIN; you lease a six-bedroom house and confirm with San Antonio's code office that six residents triggers no boarding-home permit; you bind liability and property insurance; you adopt and customize the Operations Manual; you hire one live-in house manager with two years sober and run DPS background checks; you apply to TROHN and, once accredited, register with The Center for Health Care Services OSAR (210-261-3076) and pursue a Be Well Texas subsidy partnership (888-852-3935) for eligible young-adult residents. Revenue: resident fees plus Level II/III vouchers. Timeline: business in days, home in weeks, accreditation over the following weeks to months.

You can do this

Opening a recovery residence in Texas is real work — but it's a real, walkable path, and people across the state do it every day. You don't have to be a lawyer or hire a consultant. You need a safe home, an honest operation, written policies, and accreditation — and now you have the map, one rung at a time, from your first question to your first accredited, funded bed. Verify the current details as you go, and lean on TROHN, Oxford House, and your regional OSAR — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in Texas Health & Safety Code Chapter 469 (accreditation), Chapter 464 (treatment licensing), Chapter 123 (community homes), Chapter 260 (boarding homes), the federal Fair Housing Act and EKRA (18 U.S.C. § 220), and current public program information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, funding rules, and contacts change — always verify with HHSC, TROHN, Oxford House, Be Well Texas, and qualified local counsel before you act.